This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Salbutamol 100mcg MDI and Amoxicillin 500mg capsules for the treatment of COPD Management

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for COPD Management

COPD Overview

- **Definition:** Chronic Obstructive Pulmonary Disease is a progressive condition characterised by persistent airflow limitation due to emphysema and/or chronic bronchitis.
- **Diagnosis:** Confirmed by spirometry showing FEV1/FVC <0.70; GOLD classification (GOLD 1-4) based on FEV1.
- **Aetiology:** Predominantly smoking-related; occupational/environmental exposures possible.

GOLD Classification and Exacerbations

- **GOLD 1:** FEV1 ≥80% predicted (mild); low exacerbation risk.
- **GOLD 2:** FEV1 50-79% predicted (moderate); higher exacerbation risk.
- **GOLD 3-4:** FEV1 <50% predicted (severe/very severe); high exacerbation risk.
- **Exacerbation:** Acute worsening with increased dyspnoea, cough, and change in sputum characteristics (colour, volume).

Exacerbation Assessment

- **Infective exacerbation:** Purulent sputum (yellow/green) indicates bacterial involvement.
- **Symptoms:** Increased dyspnoea, cough, sputum production, haemoptysis, fever.
- **Severity: Assess SpO2, RR, accessory muscle use; consider hospital referral if severe (SpO2 <88%, RR 25 or more, inability to speak). A respiratory rate of 25 or more excludes from both arms of this PGD.**

Pharmacological Management

- **Rescue therapy:** Short-acting beta-2 agonists for acute symptom relief (salbutamol).
- **Infective exacerbation:** Antibiotics for purulent sputum (amoxicillin for lower respiratory tract infection).
- **Oxygen therapy:** Target SpO2 88-92%; managed separately from this PGD.

Patient Group Direction

for the supply of

Salbutamol 100mcg MDI inhaler

for the treatment of

COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the

	employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute symptom relief in adults with COPD presenting with acute exacerbation or breathlessness Where the patient also meets the criteria of the amoxicillin arm, both medicines may be supplied at the same consultation. Each arm's inclusion and exclusion criteria, dose, quantity and records apply independently.
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD (documented spirometry and GOLD classification) Acute exacerbation or episodes of breathlessness requiring symptom relief Capable of using inhaler device or willing to use spacer Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to salbutamol or other beta-2 agonists Severe exacerbation with hypoxia (SpO2 <88%) requiring emergency referral and oxygen therapy Acute distress with inability to speak, cyanosis, or signs of respiratory failure Respiratory rate 25 breaths per minute or more: emergency referral. Respiratory rate must be measured and recorded before any supply; if it has not been measured, do not supply.
Cautions	Cardiovascular disease: salbutamol can increase heart rate and blood pressure; assess cardiac risk Hypertension: monitor blood pressure; beta-2 agonists may worsen Coronary artery disease or recent MI: assess risk/benefit; monitor for

	angina Diabetes mellitus: hyperglycaemia is possible. Advise a patient who self-monitors to check blood glucose more often during the exacerbation and to report readings higher than usual to the GP or diabetes team. Hyperthyroidism: beta-2 agonists can worsen symptoms Hypokalaemia: may be worsened by beta-2 agonists, particularly with diuretics, corticosteroids or theophylline. Potassium cannot be measured in the pharmacy; where the patient is known to have low potassium, inform the GP so that it can be checked. Pregnancy: generally safe; ensure informed consent
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Salbutamol 100mcg metered-dose inhaler (MDI)
Legal category	POM
Storage	Store below 25°C. Keep away from heat and direct sunlight. Shake well before each use.
Route/method of administration	Inhalation via MDI, with or without spacer
Dose and frequency	1 to 2 puffs (100 to 200 micrograms) as required for breathlessness, up to 4 times in 24 hours Use for acute symptom relief during exacerbations or ongoing dyspnoea Use spacer device to optimise drug delivery if patient is not experienced with MDI Maximum 8 puffs in 24 hours under this PGD. A need for more than this, for relief more often than every 4 hours, or for the reliever on most days, is uncontrolled or exacerbating disease: same-day GP or urgent care referral. Ten puffs through a spacer with no relief is an emergency: call 999.
Quantity to be administered and/or supplied	1 MDI inhaler (200 doses)
Maximum or minimum treatment period	One inhaler (200 actuations) per supply. Maximum 2 supplies in any 12 months under this PGD; a third request is a GP review of the

	patient's COPD, not a further supply.
Adverse effects	Common: fine tremor of hands, headache, nervousness, tachycardia, palpitations Uncommon: muscle cramps, anxiety, dizziness Rare: hypersensitivity reactions, arrhythmias, myocardial ischaemia, angina

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • respiratory rate measured at the consultation (breaths per minute) • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the rescue inhaler (salbutamol) as needed when you experience breathlessness. Do not use more than 8 puffs in 24 hours. If you need it more often than every 4 hours, or on most days, see your GP the same day. If ten puffs through a spacer give no relief, call 999.

	Use a spacer device if you have difficulty coordinating MDI actuation. Monitor your sputum colour: purulent (yellow/green) sputum suggests continued infection. Seek immediate medical attention if symptoms worsen despite treatment, or if you develop fever, persistent chest pain, or haemoptysis. Seek urgent assessment if you experience worsening breathlessness, difficulty speaking in sentences, confusion, or cyanosis. Monitor your oxygen saturation if you have a pulse oximeter at home; report any drop below 88%. Ensure you have regular follow-up with your GP to review your COPD management plan. Report any allergic reactions (rash, facial swelling, difficulty breathing) immediately.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Chronic obstructive pulmonary disease in adults
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply of

Amoxicillin 500mg capsules

for the treatment of

COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of infective acute exacerbations of COPD characterised by purulent sputum (bacterial lower respiratory tract infection) Where the patient also meets the criteria of the salbutamol arm, both medicines may be supplied at the same consultation. Each arm's inclusion and exclusion criteria, dose, quantity and records apply independently.
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD (documented spirometry and GOLD classification) Acute exacerbation with purulent sputum (yellow/green coloration indicating bacterial infection) Able to take oral medication Capable of giving valid informed consent
Exclusion criteria	Known penicillin or beta-lactam allergy Infectious mononucleosis (amoxicillin can precipitate severe rash) Severe renal impairment (eGFR <30 mL/min/1.73m2); dose adjustment needed Antibiotic resistance suspected in local resistance patterns - check local microbiology guidance Respiratory rate 25 breaths per minute or more: emergency referral. Respiratory rate must be measured and recorded before any supply; if it has not been measured, do not supply.
Cautions	Mild to moderate renal impairment (eGFR 30 mL/min/1.73m2 or above): the standard dose is suitable. Renal function is not monitored under this PGD. Hepatic impairment: generally safe. Liver function is not monitored under this PGD; advise the patient to seek medical advice if jaundice or dark urine develops during or after the course. Pregnancy and breastfeeding: amoxicillin is generally safe; ensure informed consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception Check local antibiotic resistance patterns before supply (Haemophilus influenzae, Moraxella catarrhalis, Streptococcus pneumoniae coverage) This PGD authorises amoxicillin only. Where local resistance makes amoxicillin unsuitable, do not supply under this PGD; refer to the GP

	for an alternative.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Amoxicillin 500mg capsules
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral
Dose and frequency	500mg (one capsule) three times daily To be taken on an empty stomach (1 hour before or 2 hours after meals) for optimal absorption Or may be taken with food if GI upset occurs
Quantity to be administered and/or supplied	15 capsules (5-day course)
Maximum or minimum treatment period	5 days
Adverse effects	Very common: diarrhoea, nausea Common: rash (non-allergic), vomiting, abdominal pain, vaginitis Uncommon: indigestion, headache Rare: hypersensitivity reactions (urticaria, Stevens-Johnson syndrome, TEN), hepatitis, C. difficile infection, angioedema, anaphylaxis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • respiratory rate measured at the consultation (breaths per minute)

	• name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Complete the full course of amoxicillin even if symptoms improve. Take amoxicillin at regular intervals, ideally 1 hour before or 2 hours after meals for best absorption. If using oral contraception, use additional contraceptive methods during and for 7 days after the antibiotic course. Monitor your sputum colour: purulent (yellow/green) sputum suggests continued infection. Seek immediate medical attention if symptoms worsen despite treatment, or if you develop fever, persistent chest pain, or haemoptysis. Seek urgent assessment if you experience worsening breathlessness, difficulty speaking in sentences, confusion, or cyanosis. Monitor your oxygen saturation if you have a pulse oximeter at home; report any drop below 88%. Ensure you have regular follow-up with your GP to review your COPD management plan. Report any allergic reactions (rash, facial swelling, difficulty breathing) immediately.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017

https://www.nice.org.uk/guidance/mpg2

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Valid from date	Expiry date		
30/9/26	31/7/27		

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 36: respiratory rate 25 breaths per minute or more added as an exclusion in both arms, with the respiratory rate measured and recorded before any supply and added to the records row of both arms; the guidance summary severity line now reads RR 25 or more.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm headings corrected from administration to supply; both arms are supply.

Both arms state that salbutamol and amoxicillin may be supplied at the same consultation, each on its own criteria and records.

Amoxicillin arm inclusion aligned to the salbutamol arm: documented spirometry and GOLD classification.

Cautions that asked the pharmacy to monitor potassium, blood glucose, renal function or liver function are reworded as actions a pharmacy can take.

The caution permitting wider-spectrum antibiotics is replaced: this PGD authorises amoxicillin only, and a resistance concern is a referral.

Follow-up advice is split by arm: the salbutamol arm no longer tells the patient to complete a course of amoxicillin, and it now carries the 8 puffs in 24 hours limit and the 999 threshold; the amoxicillin arm no longer carries the inhaler lines.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

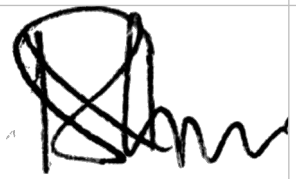
Salbutamol dose, maximum dose and maximum period stated: 1 to 2 puffs up to 4 times in 24 hours, maximum 8 puffs in 24 hours, one inhaler per supply and at most 2 supplies in 12 months, with the same-day and 999 thresholds. The previous version said '2-4 puffs as required', 'may repeat at intervals as needed' and 'ongoing', which is no maximum at all.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

GET REAL HEALTH

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.